BASELINE → TREATMENT → FOLLOW-UP

When checks happen

Relative milestones, not equal time intervals. The clinical team owns booking, result review and action on overdue tests. This is an educational map, not an automated appointment or result tracker.

Check	Before starting	Early cycles	Later cycles / transition	Next treatment / completion	Follow-up
Blood tests	full blood count (FBC), kidney and liver profile	Review before chemotherapy cycles	Check again at any phase change	Follow active medicines and abnormalities	Follow unresolved abnormalities
Echo / heart	Baseline echo (ESC); assess risk	High cardiac risk: repeat after 2 anthracycline cycles	If continuing: repeat after 4 cycles at high risk	Review cardiac symptoms and total lifetime exposure	Echo within 12 months; high risk within 3 months
electrocardiogram (ECG)	Baseline ECG in the National Cancer Control Programme (NCCP) anthracycline source; repeat for symptoms or clinical risk. An ECG does not replace an echo.	Symptoms / risk-based repeat	Review if cardiac concern	ECG and echo answer different questions	Clinical indication
Cancer imaging	Record baseline extent as appropriate	Clinical response / symptoms review	Metastatic: review about2–4 months	Arrange selected surgery / next treatment	Metastatic response plan continues
dihydropyrimidine dehydrogenase (a medicine-processing enzyme) (DPD) / markers / bone	DPD before 5-FU	No routine tumour-marker clearance	Markers only as selected adjunct in metastatic care	dual-energy X-ray absorptiometry (bone density) (DEXA) if endocrine/ bone-risk indication	Follow the endocrine and bone-health plan

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00269 v8 · official regimen · ESMO 2024 early breast cancer guideline · treatment and follow-up · ESC 2022 cardio-oncology guideline · cardiac surveillance and risk · NICE NG101 · bone health and follow-up · ESMO metastatic breast cancer guideline · 2026

THE WHOLE TREATMENT JOURNEY

FEC50: treatment and checks

Metastatic breast cancer

Fluorouracil, epirubicin and cyclophosphamide on Day 1 every 21 days while beneficial and tolerated, within the lifetime anthracycline limit. Radiotherapy must not overlap FEC.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	Early cycles	Later cycles / transition	Next treatment / completion	Follow-up
Cancer medicines NCCP 00269	Agree aim, indication and medicines	FEC50 starts	Review response, toxicity and exposure	Complete planned course / next decision	Ongoing review / subsequent plan
Surgery METASTATIC	multidisciplinary team (MDT)-selected local treatment	No automatic operation during cycles	Review response / surgical plan	Selected for local need	Pathology and recovery guide next steps
Radiotherapy NCCP 00269	Assess indication	Do not give concurrently with FEC	Arrange sequential timing	Radiation team confirms start	Review effects / follow-up
Blood tests NCCP 00269	FBC, kidney and liver profile	Review before chemotherapy cycles	Check again at any phase change	Follow active medicines and abnormalities	Follow unresolved abnormalities
Echo / heart NCCP 00269 · ESC	Baseline echo (ESC); assess risk	High cardiac risk: repeat after 2 anthracycline cycles	If continuing: repeat after 4 cycles at high risk	Review cardiac symptoms and total lifetime exposure	Echo within 12 months; high risk within 3 months
ECG NCCP 00269	Baseline ECG in the NCCP anthracycline source; repeat for symptoms or clinical risk. An ECG does not replace an echo.	Symptoms / risk-based repeat	Review if cardiac concern	ECG and echo answer different questions	Clinical indication
Cancer imaging NCCP 00269	Record baseline extent as appropriate	Clinical response / symptoms review	Metastatic: review about2–4 months	Arrange selected surgery / next treatment	Metastatic response plan continues

Care / check	Before starting	Early cycles	Later cycles / transition	Next treatment / completion	Follow-up
DPD / markers / bone NCCP 00269	DPD before 5-FU	No routine tumour-marker clearance	Markers only as selected adjunct in metastatic care	DEXA if endocrine/bone-risk indication	Follow the endocrine and bone-health plan
Endocrine / bone BONE · EARLY	Confirm hormone receptors	Plan endocrine therapy when appropriate	Separate from this chemotherapy prescription	Start/continue per MDT; DEXA when indicated	Risk-based bone review

What each check is for

1 Before first dose

Baseline

Confirm DPD testing before first 5-FU; interpret uracil results cautiously in renal impairment. Keep the result available when treatment changes. Baseline FBC, kidney and liver tests; repeat before each chemotherapy cycle.

Ask / act: Confirm tests and consent before prescribing.

Sources: NCCP 00269

2 Heart-monitoring plan

Baseline → cycles → after treatment

ESC supplementary guidance: baseline echo before anthracycline treatment. High/very-high cardiac risk: echo every two anthracycline cycles (for example before cycles 3 and 5 when continuing), and within 3 months after completion. All adults: echo within 12 months after completion. Other repeat tests depend on risk, cumulative exposure and symptoms.

Ask / act: Name the team responsible for booking and reviewing results.

Sources: NCCP 00269 · ESC

3 Treatment-phase review

Before each transition

Fluorouracil, epirubicin and cyclophosphamide on Day 1 every 21 days while beneficial and tolerated, within the lifetime anthracycline limit. Radiotherapy must not overlap FEC.

Ask / act: Confirm active medicines, support script and the next test checkpoint.

Sources: NCCP 00269

4 Response and follow-up

Planned review points

Record disease sites before treatment. Response imaging is generally considered every 2–4 months under ESMO guidance, adjusted to symptoms and disease pace. For a 21-day course this is approximately after 3–6 cycles; the team chooses the scan and exact checkpoint. New symptoms may need earlier assessment.

Ask / act: Review symptoms and imaging together; do not change treatment on tumour markers alone.

Sources: NCCP 00269

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Sources checked 2026-10-23 · Source reconciled; independent clinical, pharmacy and patient/PPI review pending · SACTCheck v0.84.17